Clinical Study Summary Report (CSSR) Document Status: Final | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Study to Learn About the Effects of the Combination of Elranatamab (PF-06863135) and Iberdomide in Patients With Relapsed or Refractory Multiple Myeloma (MagnetisMM-30) **Short Title/Acronym:** MagnetisMM-30 **Protocol Number:** PF-06863135 **NCT Number:** NCT06215118 **Trial Status:** Recruiting **Sponsor Name:** Pfizer **Investigational Product:** Elranatamab (BCMA-CD3 bispecific antibody) and Iberdomide (CELMoD agent) **Phase of Development:** Phase 1 (Phase 1b) **Medical Monitor:** Pfizer Clinical Operations Lead / Clinical Trials Navigator

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: * Part 1: To evaluate safety and tolerability by determining Dose-Limiting Toxicities (DLTs) during the first cycle to identify the recommended doses for the combination. * Part 2: To evaluate Adverse Events (AEs) and lab abnormalities for dose optimization. **Secondary Objectives:** To evaluate the Overall Response Rate (ORR), Complete Response Rate (CRR), Minimal Residual Disease (MRD) negativity rate, time-to-event outcomes (such as Progression-Free Survival [PFS] and Overall Survival [OS]), pharmacokinetics, and immunogenicity.

2.2 Background & Rationale To address the clinical need for deep and durable responses in patients with Relapsed or Refractory Multiple Myeloma (RRMM). Elranatamab has shown efficacy as a single agent. Iberdomide is a novel CELMoD agent that enhances antimyeloma tumoricidal and immunomodulatory activity. The synergistic combination of these two mechanisms of action may provide additional, potent clinical benefit for patients with RRMM whose disease has failed standard lines of therapy.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Dose-Finding (Single-arm in Part 1 dose escalation; Randomized in Part 2 dose optimization) **Blinding:** Open-label **Randomization:** 1:1 Randomization in Part 2 (stratified by the number of prior lines of therapy [1 vs >1])

3.2 Planned Enrollment & Duration Target N: 87 patients **Number of Sites:** Multicenter (Global, including sites like Memorial Sloan Kettering Cancer Center) **Estimated Study Start:** January 2024 **Estimated Primary Completion:** August 2026

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Prior diagnosis of multiple myeloma as defined by IMWG criteria. 2. Measurable disease based on IMWG criteria as defined by at least 1 of the following: Serum M-protein ≥ 0.5 g/dL by SPEP, Urinary M-protein excretion ≥ 200 mg/24 hour by UPEP, or Serum immunoglobulin FLC ≥ 10 mg/dL (≥ 100 mg/L) AND abnormal serum immunoglobulin kappa to lambda FL ratio (<0.26 or >1.65). 3. Received prior lines of therapy for multiple myeloma, consisting of at least 1 immunomodulatory drug and 1 proteasome inhibitor (Part 1: 2-4 prior lines; Part 2: 1-3 prior lines). 4. ECOG performance status 0-1. 5. Resolved acute effects of any prior therapy to baseline severity or CTCAE Grade ≤ 1.

4.2 Exclusion Criteria 1. Diagnosis of plasma cell leukemia, smoldering multiple myeloma, Waldenström's macroglobulinemia, amyloidosis, or POEMS Syndrome. 2. Prior therapy targeting BCMA or CD3 redirecting therapy, or prior treatment with iberdomide (CC-220) or mezigdomide. 3. Stem cell transplant within 12 weeks prior to enrollment or active graft-versus-host disease (GVHD). 4. Impaired cardiovascular function or clinically significant cardiovascular diseases. 5. Active, uncontrolled bacterial, fungal, or viral infection. 6. Any other active malignancy within 3 years prior to enrollment, except for adequately treated basal cell or squamous cell skin cancer, or carcinoma in situ. 7. Administration of strong inhibitor or inducer of CYP3A4/5 within 2 weeks prior to dosing and during the study. 8. Administration with an investigational product within 30 days preceding the first dose of study intervention. 9. Participant is unable or unwilling to undergo protocol required thromboembolism prophylaxis.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Elranatamab is given as step-up priming doses followed by a full dose (dosing interval scales from weekly to Q2W, then Q4W depending on dose level). Iberdomide is taken once a day for 21 days of each 28-day cycle. **Route of Administration:** Elranatamab via Subcutaneous (SC) injection; Iberdomide via Oral administration. **Duration of Treatment:** 28-day cycles, continuing until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for RRMM complications and infection prophylaxis. **Prohibited:** Other concurrent systemic anti-myeloma therapies or any other investigational drugs.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, IMWG criteria disease measurement, Labs | | **Baseline** | Day 1 (Cycle 1) | Step-up dosing of ELRA, First dose of IBER, PK/safety labs | | **Interim** | Day 1 of subsequent Cycles | Safety monitoring (DLTs, CRS tracking), Efficacy assessments (SPEP/UPEP/FLC), PK draws | | **End of Study** | At Progression / Toxicity | Final physical exam, Exit Interview, IP Accountability, Survival/PFS Follow-up |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Part 1:** Number of participants with dose limiting toxicity (DLT). * **Part 2:** Number of participants with Adverse Events (AE) by Seriousness and Relationship to Treatment.

7.2 Secondary & Exploratory Endpoints * **Part 1:** Number of participants with Adverse Events (AE) by Seriousness and Relationship to Treatment. * **Part 1 and Part 2:** Number of Participants with Adverse Events (AE) characterized by type, frequency, severity. * **Part 1 and Part 2:** Number of Participants with Clinically Significant Change from Baseline in Laboratory Abnormalities. * Overall Response Rate (ORR) and Complete Response Rate (CRR) per IMWG criteria. * Time-to-event outcomes including Progression-Free Survival (PFS) and Overall Survival (OS). * Minimal Residual Disease (MRD) negativity rate and Pharmacokinetics (PK).

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard reporting emphasizing monitoring for Cytokine Release Syndrome (CRS), Immune effector cell-associated neurotoxicity syndrome (ICANS), neutropenia, and infections. **Serious Adverse Events (SAEs):** Reported within standard 24-hour timelines; particular scrutiny on Grade 3/4 events such as febrile neutropenia and severe CRS. **Data Monitoring Committee (DMC):** Dose escalation guided by Bayesian Optimal Interval (BOIN) design, overseen by an internal safety review committee.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) and Safety Analysis Set (all patients receiving ≥ 1 dose of study drug). **Sample Size Justification:** Target of 87 subjects to adequately power safety, DLT incidence via BOIN statistical model, dose optimization, and comparative AE rates. **Handling of Missing Data:**

Standard survival analysis algorithms (e.g., Kaplan-Meier) used for time-to-event endpoints; censoring at last known alive date.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote source data verification by sponsor clinical research associates (CRAs). **Audit Trail:** Data entered into a validated electronic Data Capture (EDC) system with full audit trails and electronic queries.

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11. Study Identifiers & Governance

Primary ID: PF-06863135 **Registry Number:** NCT06215118 **Sponsor/Lead Organization:** Pfizer **Study Title:** MagnetisMM-30 **Official Regulatory Title:** A PHASE 1B, OPEN-LABEL STUDY OF ELRANATAMAB IN COMBINATION WITH IBERDOMIDE IN PARTICIPANTS WITH RELAPSED REFRACTORY MULTIPLE MYELOMA

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12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Multiple Myeloma (Relapsed or Refractory) **Diagnosis Threshold:** Measurable disease per IMWG criteria **Medical Methodology:** Bispecific Antibody (BCMA-CD3) + CELMoD agent synergistic therapy **Optimization Target:** Recommended Phase 2 Dose (RP2D) identification & optimization

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13. Study Procedures & Methodology

Management Pathway: Standard oncological pathway for RRMM response tracking (SPEP, UPEP, bone marrow biopsies) **Education Protocol (HCP):** Safety management training for Cytokine Release Syndrome (CRS) and ICANS mitigation **Education Protocol (Patient):** Injection site reaction management and oral adherence counseling **Quality Audit Mechanism:** DLT review committee & BOIN algorithmic oversight

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14. Observation & Follow-Up Schedule

Total Duration: Until disease progression or unacceptable toxicity (up to approx. 2 years active follow up) **Onsite Visit Cadence:** Step-up dosing days, then Day 1 of every 28-Day Cycle **Remote Monitoring Frequency:** Routine telemedicine outreach for AE tracking between cycles **Checklist Review Interval:** Continuous PK and disease response assessments (per cycle)

15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]